

- Late-stage patients file 11–22 insurance claims during treatment
- Deduction rates for cancer claims have risen by 58% in three years
- Over 55 million Indians are pushed below the poverty line each year due to catastrophic health spend, of which cancer is the major chunk of expenditure

WHY METASTATIC CANCER PATIENTS GO UNCOUNTED

- India counts only incidence and mortality, not lived metastatic burden; the National Cancer Registry Programme covers only 15-18% of the population
- Many late-stage patients treated in private or smaller hospitals are never recorded
- For some advanced cancers like lung or stomach, median survival in Indian cohorts is barely a year—not enough for them to enter or remain in national datasets
- When families stop hospital-based treatment once disease becomes advanced, patients are marked as "lost to follow-up"
- Death certificates often only specify lung or breast cancer, without saying it was metastatic, masking severity and treatment burden

for advanced cancer and standardise cancer-care riders that cover essential therapies without co-payments.

Pain and punishment

One reason metastatic cancer remains invisible in policy is the absence of data. India lacks a precise count of how many people are living with metastatic cancer. People who arrive at hospitals already metastatic, or who stop active treatment and shift to palliative care, tend to fall out of formal datasets. The same happens with patients who present with metastases at diagnosis, but later discontinue active treatment and move to palliative care. The National Cancer Registry Programme tracks diagnoses and deaths but does not follow patients when disease progresses from early stage to stage IV.

"Once their disease is no longer curable," says Dr Ishita Gandhi, medical director of CanSupport, "cancer patients largely disappear from registries and programme metrics. Symptom burden, quality of life and place of care are not measured, and what we do not measure, we do not invest in."

As a result, many patients with metastatic cancer die in excruciating pain because palliative care tends to arrive only in the final weeks of life, instead of being integrated at diagnosis. "Cancer pain is treatable," says Dr Chanana. "Yet many patients still die in pain because the help does not reach them in time."

The pain of metastatic disease, says Dr Jayachandran P.K., consultant in Medical Oncology at Apollo Cancer Centre, Chennai, evolves over time. "In advanced Stage IV cancer, pain becomes chronic and multifactorial: from bone metastases, tumour invasion into organs, or nerve damage. But the psychological pain is just as deep. Patients face fear of abandonment, loss of control, and anxiety about the unknown."

While complete pain elimination may not always be possible, integrated palliative care can make pain manageable through opioids, nerve blocks, targeted radiation, and counselling.

The morphine shortage

Relieving suffering is a moral obligation of the health professionals, says Dr M.R. Rajagopal, chairman emeritus of Pallium India, the largest palliative care organisation in India. "Pain or other symptoms like breathlessness can be beyond one's imagination. We have the means to treat them at low cost, and not doing it is passive violence," he adds. According to him, even though the 2014 amendment to the Narcotic Drugs and Psychotropic Substances Act placed morphine in the new category of "essential narcotic drugs", and replaced multiple licences with a single, simplified approval for recognised hospitals, many state-level barriers still prevent it from reaching those who need it most.

Dr Chanana sees this daily. "India produces medical morphine," he says. "Yet many patients with advanced cancer endure excruciating pain because their local hospitals cannot stock it." It is what happened to Manoj Rao. His family travelled nearly 100 kilometres to find a pharmacy that could dispense it. "That first night on morphine felt unreal," Rao recalls. "I had forgotten a body could rest."

Vishal Lathwal, CEO of Apollo Home Healthcare, points to another strain: caregiving. Shrinking families are struggling to support patients at home, often at the expense of their own physical and emotional health. "Nearly 70 per cent of advanced cancer patients need palliative support, yet structured options remain limited," he says. Clearer end-of-life guidelines and credible homecare models, he believes, could help patients get coordinated, dignified care outside hospital walls.

Living with cancer is hard enough. Being denied relief from avoidable suffering makes it far worse. ■